

SHOULDER (SUBACROMIAL) INJECTION — One-Pager

1 · MATCH SPACE TO DIAGNOSIS

Diagnosis	Inject
RC tendinopathy / subacromial bursitis / impingement	Subacromial (this lesson)
Glenohumeral OA · adhesive capsulitis	Glenohumeral (intra-artic.)
AC osteoarthritis	AC joint

2 · EXAM & MIMICS (+LR)

Finding	Suggests	+LR
+Neer/Hawkin, painful arc	Impingement/RC	—
Drop-arm +	Full-thick RC tear	spec.
Speed/Yergason + groove TTP	Biceps tendonitis	—
Global active+passive ROM loss	Adhesive capsulitis / GH OA	—
Spurling +	Cervical radiculopathy	4.5
↓ biceps reflex	Cervical radiculopathy	9.1

Don't inject a neck: negative shoulder tests + Spurling/↓ biceps reflex → **MRI C-spine**, not a shoulder injection.

3 · WHAT WORKS (GRASP/CSAW)

- Subacromial CSI: better at **~8 wk, no diff 6–12 mo** (GRASP Lancet 2021)
- **Progressive exercise = core**; 1 PT session not inferior to 6; injection improves adherence
- Subacromial decompression: little benefit vs conservative (CSAW Lancet 2018)
- **Full-thick RC tear** + failed 3 mo PT → surgery better at 2 yr (Cederqvist 2021); partial-thick: PT = surgery

4 · CONTRAINDICATIONS

Defer	Overlying skin infection; surgery anticipated <3 mo (CSI <4 wk pre-arthroscopy → infection); CSI same space <3 mo ago
NOT contraindic.	Well-controlled DM (counsel glucose); prior surgery w/o intra-articular hardware

5 · TECHNIQUE (posterolateral)

- Mark scapular spine + lateral acromion → **posterolateral point of acromion**; insert **~1–2 cm below** in soft recess.
- Chlorhexidine scrub ×30 s; stabilize shoulder w/ non-dominant hand.
- Needle **under acromion, slightly cephalad toward coracoid**, ~3 cm.
- Aspirate (no blood) → inject 1% lido + triamcinolone 40 mg **WITHOUT resistance**.

Resistance = in tendon (not bursa) → injecting risks tendon rupture; withdraw & redirect under acromion.

6 · COUNSELING & F/U

- Set expectations: **short-term relief**; exercise = durable benefit.
- Diabetics: transient glucose rise.
- Refer PT; begin within ~4 wk. Return precautions: signs of infection.

GH OA WITHOUT TRAUMA?

Shoulder is non-weight-bearing → primary GH OA uncommon w/o trauma/RC injury. Consider metabolic cause (**hemochromatosis** — iron/TIBC/ferritin); RA in differential.

SUPPLIES

1% lido (5 cc) · triamcinolone 40 mg/mL (1 cc) · alcohol swabs · 5–10 cc syringe · 18 g (draw-up) + 21–25 g 1.5" (inject) · marker · chlorhexidine · gauze · band-aid.

Adapted from TeachIM.org Shoulder (subacromial space) Injection (Fainstad, Brett, Thompson, Melton; Feb 2021, rev. 2023/2025), CC BY-NC 4.0 — non-commercial educational use. Optimized June 2026; currency verified (AAOS 2019 RC guideline; GRASP/CSAW/UK FROST/Cederqvist RCTs current). Clinician-review before teaching. Not medical advice.